

After implant surgery

Protect the wound, keep pressure off the implants and support healing

This information applies after implant placement, implant uncovering, bone grafting and soft tissue grafting. Aftercare depends on the extent and location of your operation. **Your individual treatment plan takes priority.**

Do not bite on the freshly operated ridge.

Do not chew solid food directly on the surgical site. Pressure and repeated movement can disturb wound closure, a bone graft and implant healing. Protect the area even if it does not hurt. Do not test implants or stitches with your tongue, fingers or other objects.

Eat without putting pressure on the surgical site

Wait until numbness has completely worn off before eating; do not chew or have hot drinks while still numb. Then choose soft, cool or lukewarm foods such as mashed potatoes, scrambled eggs or soft vegetables. Hard crusts, nuts and firm raw foods can put pressure on the wound. Drink enough fluids and choose nutritious food with sufficient protein.

If possible, chew only on the **side that was not operated on**. If both sides were treated or you cannot avoid the area, choose puréed or very soft food requiring no chewing pressure. Protection does not automatically end after one week or when stitches are removed: **wait for our clearance**.

Dentures and temporary teeth must not press on the wound

Wear an existing denture only after we have checked it, adjusted it if needed and approved its use. Even an apparently comfortable denture can press on the surgical site. We will tell you whether and when a removable replacement may be worn.

If there is a pressure sore or anything feels loose, stop chewing with it and contact us promptly. Remove a removable denture that presses on the wound unless instructed otherwise. Do not loosen or adjust fixed temporary teeth yourself. Immediately fitted implant teeth require a separate loading and diet plan.

The first few hours

Rest with your upper body raised. Cool the area from outside for about ten minutes, then pause. Use a cloth to protect the skin; do not apply ice directly or put pressure on the site. Do not drive after local anaesthesia; public transport is generally possible. After sedation or general anaesthesia, collection from the practice and supervision are required. For at least 24 hours, avoid driving, machinery, important decisions and alcohol; follow any longer restrictions from the anaesthetic team.

Oral hygiene while the wound heals

A clean mouth helps healing, but the fresh wound also needs protection from physical irritation. Ask us to show you which areas you may clean.

- **Other teeth:** brush gently twice daily. Do not leave the rest of your mouth uncleaned because of the operation.
- **Wound and stitches:** do not brush them directly at first. We will explain when and how to start at your review. The immediate surgical site is often left unbrushed during the first week, sometimes longer after grafting.
- **Once approved:** use a soft or surgical toothbrush carefully. Exposed healing caps need their own cleaning plan.
- **Avoid:** water flossers, irrigation syringes, floss or interdental brushes around the fresh wound unless specifically instructed. Do not pull stitches, scrape off wound coatings or stretch the lip to inspect the sutures.

Antiseptic mouthwash

Do not rinse vigorously or spit forcefully on the day of surgery. A prescribed antiseptic rinse is usually started **the following day**, gently bathing the area. Follow the prescribed product and your personal schedule.

Example only if prescribed: alcohol-free chlorhexidine 0.2%, 10 ml twice daily, held gently in the mouth for about one minute, then allowed to drain out carefully. Treatment is usually limited to a short course, for example 7–14 days; we specify the duration. Other concentrations may require a different dose.

Do not swallow or rinse with water afterwards. Wait at least 30 minutes before eating or drinking. Use it at a different time from toothbrushing because toothpaste ingredients may reduce its effect; follow the product leaflet.

Mouthwash does not replace brushing and should not be used indefinitely. Temporary staining, altered taste and irritation may occur. Stop and seek advice if you do not tolerate it; call 112 for breathing difficulty or severe allergic swelling. Tell us about any known chlorhexidine allergy.

Extra precautions after bone grafting or a sinus lift

Do not touch or remove stitches or an exposed membrane. Contact us for review if the wound opens, graft material becomes visible or there is noticeable loss of graft particles. Do not try to clean the site yourself.

After a sinus lift: do not blow your nose or sniff forcefully for at least one week; sneeze with your mouth open. Avoid flying and diving for at least two weeks, longer after extensive surgery. Follow your personal clearance and any prescribed nasal medication.

Pain relief and medicines

The table gives our **adult practice protocol after individual approval**. Body weight, pregnancy, stomach, kidney or liver disease and other medicines may require lower doses or different medication. Children and adolescents need their own plan based on age and weight.

Medicine	Dose if suitable	Note
Ibuprofen 600 mg	Every 6–8 hours, max. 3 doses/day	Take with food
Paracetamol 500–1000 mg	Alternatively/additionally every 6 hours	Max. 4 g/day; not suitable for all body weights
Novalgin drops	Up to 30 drops 4 times/day, only if prescribed	Only at 500 mg/ml and 20 drops/ml
No additional aspirin/ASA	Avoid as a painkiller	Do not independently stop prescribed long-term aspirin

If fever, chills, sore throat or painful mucosal sores develop while taking Novalgin/metamizole: **stop it immediately and seek urgent medical assessment**. Do not add paracetamol-containing combination products without advice.

Do not combine two anti-inflammatory painkillers such as ibuprofen and diclofenac. Take antibiotics only if prescribed and exactly as instructed; do not use leftovers. Contact us for a rash, severe diarrhoea or other significant side effects. Do not stop prescribed blood thinners, including aspirin, yourself.

What to expect during healing

- **First few days:** soreness, swelling, bruising and slightly bloodstained saliva can occur. Swelling often peaks on the second or third day and should then improve.
- **Review:** attend your arranged appointment even if you feel well. Stitches are usually removed after 1–2 weeks; a later removal may be appropriate after larger grafts.
- **Further healing:** a closed gum surface does not mean that the implant is ready for full chewing forces. Implant healing often takes several months, and grafting may extend this. We examine the area before approving further loading.

Supporting recovery

Exercise and heat: avoid sport, heavy lifting and strenuous activity for at least one week. Larger grafts may require a longer break. Avoid saunas, hot baths and intense heat during early healing; resume according to individual advice.

Nicotine and alcohol: do not smoke or vape for at least four weeks; stopping permanently is preferable. Smoking increases the risk of poor wound healing and implant loss. Avoid alcohol during early healing, particularly while taking painkillers or antibiotics or after sedation.

General health: keep diabetes well controlled and maintain food and fluid intake. Contact us early if eating or taking your regular medication becomes difficult.

Warning signs and what to do

Contact us promptly, immediately for severe symptoms: persistent or heavy bleeding, worsening pain or swelling after initial improvement, fever, pus, an unusual smell with symptoms, wound opening or a denture pressing on the wound.

Loose healing cap, screw or replacement teeth: stop loading the area, do not tighten anything yourself and contact the practice. Keep any part that has already fallen out; do not put it back yourself. If numbness of the lip, chin or tongue persists after the anaesthetic should have worn off, contact us the same day.

If bleeding occurs

Sit upright, stay calm and avoid vigorous rinsing. If shown how, place clean gauze directly over the accessible bleeding wound and maintain steady pressure for about 20–30 minutes. **This targeted pressure to stop bleeding is an exception; do not bite on an implant, graft or temporary teeth.** If you are unsure where to apply pressure or the bleeding does not stop, contact us immediately. Do not fall asleep with gauze in your mouth.

Contact us promptly for warning signs

Practice: **06151 786540**. After surgery outside practice hours: **+49 151 10437450**. If unavailable: dental emergency service **01805 60 70 11**.

For a life-threatening emergency, breathing difficulty, severe difficulty swallowing or rapidly increasing swelling of the floor of the mouth, neck or tongue: **call 112 immediately**.

Your personal aftercare plan

Operated area / special instructions: _____

Pressure avoidance and diet until: _____

Denture / temporary teeth allowed from: _____

Mouthwash / dose / start / end: _____

Review / suture removal: _____

After healing: home implant care, professional cleaning and regular dental and surgical reviews remain important. Read more: [Open implant review information ↗](#).



Your aftercare information online

patienteninfo.mkg-im-carree.de/implantat-nachsorge-en/

Scan the QR code with your phone.

Clinical sources: Guy's and St Thomas': After having a dental implant; Cambridge University Hospitals: Dental implants; CUH: Bone grafting; NHS: Chlorhexidine. Accessed 25 September 2026. Adapted to MKG im Carree's individual practice protocol. This information does not replace a personal assessment.